

D13 — IR Post-Procedure Note

Dr. F. Alvarez, Interventional Radiology | Interventional Radiology | 02/22/2026 16:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

INTERVENTIONAL RADIOLOGY POST-PROCEDURE NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/22/2026 16:00 | Provider: Dr. F. Alvarez, Interventional Radiology

PROCEDURE: Selective angiographic embolization of left gastric artery

INDICATION: Recurrent upper GI hemorrhage from gastric ulcer after failed endoscopic hemostasis

TECHNIQUE:
Right common femoral artery access. 5F sheath placed. Celiac arteriography performed demonstrating active extravasation from a branch of the left gastric artery corresponding to the known gastric ulcer location. Selective catheterization of the left gastric artery performed with microcatheter. Embolization with gelatin sponge (Gelfoam) pledgets until cessation of flow to the bleeding branch confirmed on post-embolization angiography. Adequate collateral flow to the gastric fundus confirmed. Sheath removed, hemostasis achieved with manual compression.

FINDINGS: Active extravasation from left gastric artery branch – successfully embolized.

COMPLICATIONS: None.

- POST-PROCEDURE ORDERS:
1. Bed rest x 6 hours, right groin site check Q15min x 1 hour, then Q1H x 4 hours
 2. Monitor Hgb Q6H x 24 hours
 3. NPO x 12 hours, then advance diet as tolerated
 4. Hold anticoagulation minimum 5 days post-procedure (restart no sooner than 02/27/2026)
 5. Groin site: monitor for hematoma, distal pulses
 6. Continue IV pantoprazole drip

Transfer to MICU for hemodynamic monitoring.

Electronically signed by: Dr. F. Alvarez, MD
Date: 02/22/2026 16:45

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